

Common children's respiratory conditions

A first-year-friendly guide to the patterns you are most likely to meet on placement: bronchiolitis, croup, viral wheeze or asthma, and pneumonia. Simple language, clear red flags, and practical nursing actions.

4

common conditions

1

simple way to
compare them

A–E

placement-ready
thinking

**Red
flags**

you should not miss

1 Start with what you can see and hear

First-year students do not need the perfect diagnosis in the first sentence. Start with a simple approach: what noise is the child making, how hard are they working to breathe, and are they tiring or feeding poorly?

LISTEN

Stridor usually means upper airway narrowing. **Wheeze** is more often lower airway.

Crackles can sit with bronchiolitis or pneumonia.

LOOK

Check **recession, respiratory rate, tracheal tug, nasal flaring, grunting, colour, alertness**, and whether the child can **feed or talk**.

THINK

Ask yourself: is this an **upper airway** problem, a **lower airway** problem, or a child who looks generally unwell and needs urgent review?

CLUE	WHAT IT OFTEN SUGGESTS	WHY IT MATTERS TO NURSES
Barking cough + stridor	Croup / upper airway narrowing	Keep the child calm and watch closely for worsening airway compromise
Expiratory wheeze	Viral wheeze or asthma pattern	Think spacer, response to treatment, and local wheeze pathway
Infant + coryza + poor feeding	Bronchiolitis pattern	Feeding, hydration, oxygenation, and fatigue matter as much as chest sounds
High fever + focal crackles	Pneumonia	Think infection, sepsis awareness, hydration, and escalation if very unwell

RED FLAGS

- Looks blue or pale
- Apnoea or severe tiredness
- Stridor at rest
- Grunting or marked recession
- Poor feeding with worsening work of breathing
- Hard to wake or less responsive

PLACEMENT SENTENCE STARTER

"I'm worried because this child's breathing looks harder work, and I want to escalate for review while we keep reassessing oxygenation, observations, and feeding."

2

Compare the common conditions

This is the simple level first-year nurses need: pattern recognition, not specialist respiratory medicine.

CONDITION	TYPICAL PATTERN	COMMON SOUND	WHAT YOU SHOULD NOTICE	FIRST NURSING THOUGHTS
Bronchiolitis	Usually infants, often starts like a cold, then feeding and breathing worsen	Wheeze and/or crackles	Poor feeding, recession, fatigue, low saturations	Supportive care, oxygenation, hydration, feeding support, escalation if tiring
Croup	Barking cough, hoarse voice, worse at night	Stridor	Is the stridor only when upset, or also at rest? How distressed does the child look?	Keep calm, minimise upset, urgent review if stridor at rest or severe distress
Viral wheeze / asthma	Wheezy child, often previous episodes or triggers, may improve with treatment	Expiratory wheeze	Breathlessness, prolonged expiration, response to inhaler/spacer	Think local wheeze pathway, inhaler/spacer, and close reassessment
Pneumonia	Fever, cough, increased work of breathing, child may seem more generally unwell	Sometimes crackles, sometimes reduced air entry	High fever, focal chest findings, lethargy, hydration status	Think infection, fluids, sepsis awareness, antibiotics if prescribed, escalation if unwell

KEEP IT SIMPLE

Stridor = think upper airway.

Wheeze = think lower airway.

Crackles + high fever = think pneumonia more strongly.

USEFUL REMINDER

Not every preschool child with wheeze has asthma. Viral-induced wheeze is common, especially during viral illnesses. History and pattern matter.

3

First-year nursing actions

These are safe, high-level first thoughts. Always follow your local policy, pathway, PEWS, and escalation process.

BRONCHIOLITIS

Think **supportive care**. Recheck observations, watch feeding, count wet nappies, keep the child comfortable, and escalate if work of breathing or oxygenation worsens.

CROUP

Think **upper airway**. Keep the child calm and with their parent if possible. Do not upset them unnecessarily. Escalate quickly if there is **stridor at rest** or severe distress.

VIRAL WHEEZE / ASTHMA

Think **response to treatment**. Use the wheeze/asthma pathway, check inhaler or spacer use, and reassess breathing after treatment.

PNEUMONIA

Think **infection and hydration**. Monitor observations, encourage/track fluids, watch for deterioration, and escalate if the child looks septic or more unwell.

Look

How hard is the child working to breathe? Are they tiring? Can they feed or speak normally?

Listen

Is it stridor, wheeze, crackles, or "just" noisy breathing? The sound helps guide your thinking.

Measure

Use RR, sats, temperature, heart rate, PEWS, and fluid balance to support what you are seeing.

Escalate

If the child looks worse than expected, escalate early. You do not need to wait until you are sure.

WHAT NOT TO DO

Do not get stuck trying to sound clever. On placement, simple and safe is better: describe what you see, explain why you are concerned, and escalate early.

GOOD HANDOVER PHRASE

"This child has worsening work of breathing and I'm concerned they need urgent review. Their sats are low, they are feeding poorly, and they look more tired."

4

What to say on placement

Students often know the signs but freeze on the wording. Keep your language calm, clear, and practical.

SIMPLE SBAR

Situation: "I'm concerned about a child with worsening breathing."

Background: "They have had cough/coryza/fever and are now feeding less."

Assessment: "They have recession, increased RR, and I'm worried they are tiring."

Recommendation: "Please review urgently while we continue observations and supportive care."

PARENT-FRIENDLY LANGUAGE

"Your child is working quite hard to breathe, so we're watching them closely."

"The important things for us are breathing, oxygen levels, and whether they can drink enough."

"If they become more sleepy, more blue, or breathing gets harder, tell us straight away."

FOR FIRST YEARS

- Describe what you can see before trying to explain everything.
- Use "I'm concerned because..." in handover.
- Feeding matters in infants.
- Escalating early is good practice, not failure.

SAFETY NOTE

This guide is for revision. It does not replace trust policy, PEWS, local respiratory pathways, medicines guidance, or senior clinical judgement.

SOURCES CHECKED FOR THIS VERSION

NICE bronchiolitis guidance, NICE pneumonia guidance, NHS croup, NHS asthma, and NHS England Martha's Rule. This is a high-level student guide, so treatment detail is intentionally simplified.